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SciScore Report

Below you will find your SciScore report containing three tables. Your score is calculated based on adherence to scientific rigor criteria (Table 1) and identification of key biological resources (Table 2). Table 3 contains statistical tests and oligonucleotides but is not scored. If SciScore makes any mistakes, please contact us to help us learn and improve.

Table 1: Rigor Adherence Table

Ethics
Consent: Patients aged > 18 years who gave written informed consent (patient or proxy) were included if they were: (i) at risk for PU development based on Braden risk assessment
IRB: Ethics Approval was received from both central (Ghent University Hospital) and local ethics committees for the trial protocol, informed consent forms and other relevant documents.
Inclusion and Exclusion Criteria
Patients aged > 18 years who gave written informed consent (patient or proxy) were included if they were: (i) at risk for PU development based on Braden risk assessment
Attrition
The safety population (n = 1077) was calculated after exclusion of patients in the ITT population who wanted their data excluded (n = 1), were not randomized (n = 46), did not receive at least one dressing (n = 10) or received only SOC (n = 546).
Sex as a biological variable
not detected.
Subject Demographics
Age: Patients aged > 18 years who gave written informed consent (patient or proxy) were included if they were: (i) at risk for PU development based on Braden risk assessment
Randomization
Patients and methods Study design This was a pragmatic, multicentre, randomized (1 : 1 : 1 allocation), open-label, parallel-group medical device trial performed in eight hospitals (three university or teaching hospitals and five general hospitals) in Belgium, including patients on both ICU and non-ICU wards, with patients in ICU limited to < 25.
Blinding
Randomization and blinding Patients were centrally randomized to study groups based on a 1 : 1 : 1 allocation: experimental group 1 (Allevyn(r) brand) , experimental group 2 (Mepilex(r) brand) and the control group (SOC) .
Power Analysis

Participants The sample-size calculation was based on the results of several randomized trials with an incidence of 6 of PU category 2 or worse on the sacrum, greater trochanter and heels in the standard of care (SOC) group, 4 and the treatment group having a 50 reduction in incidence of PUs of category 2 or worse. 2 , 22 To have 80 power to detect a reduction in PU incidence from 6 to 3, data had to be available for 1578 patients in total, of whom 526 had to be allocated to the control group and 1052 to the treatment group.	
Replication	
not required.	
Data Information	
Availability: Full inclusion and exclusion criteria are available in the clinical study protocol (Appendix S1 see Supporting Information) .	
Protocol Information	
Identifiers: The study was registered at ClinicalTrials.gov (NCT03442777).	NCT03442777 Warning: study registration date February 16, 2018 is after study start date February 8, 2018

Table 2: Key Resources Table

Your Sentences	REAGENT or RESOURCE	SOURCE	IDENTIFIER
Software and Algorithms			
Statistical analyses were performed using the SAS statistical package, version 9*4 (IBM, Armonk, NY, USA).	SAS		

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For a full description of scored criteria and tips for improving your score, please see https://www.scicrunch.com/sciscorereport-faq